SERIOUS VS NON-SERIOUS CASE ASSESSMENT REPORT

Pharmacovigilance Training Assignment | Synthetic case review | India regulatory context

Purpose. To assess five distinct patient safety case records against regulatory seriousness criteria, assign Serious or Non-Serious status, and identify the applicable Indian marketing authorisation holder (MAH) reporting timeline.

Source pack. pv_task4_classification_pack(1).zip. The archive contains a README, one CSV dataset with 25 synthetic case records, and a synthetic product reference CSV. Five representative records were selected for this report: SYN-PV-005, -008, -010, -013 and -024. All patient identifiers and scenarios are fictional.

1. Regulatory assessment framework

A case is generally considered **serious** if the adverse event results in any of the following: death; an event that placed the patient at immediate risk of death; inpatient hospitalisation or prolongation of existing hospitalisation; persistent or significant disability/incapacity; congenital anomaly/birth defect. An additional regulatory category is an **other medically important event** that may jeopardise the patient or require medical/surgical intervention to prevent a serious outcome.

Important distinction: seriousness is not the same as intensity/severity. A severe symptom is not automatically serious unless a seriousness outcome/criterion is met. Conversely, an event may be serious even if its symptom intensity is not described as severe.

2. India reporting timeline applied

For MAHs under the cited CDSCO 2024 PSUR guidance: **all serious adverse events: within 15 calendar days** of receipt of information; **all non-serious adverse events: within 90 calendar days** of receipt of information. These timelines apply to MAH reporting to CDSCO and NCC-PvPI. Healthcare professionals should report suspected ADRs promptly through PvPI; the MAH-specific 15/90-day schedule should not be interpreted as permission to delay initial clinical reporting.

3. Five-case classification matrix

Case / patient	Suspect product and reaction	Death / life-threatening	Hospitalisation	Disability	Other medically important event	Final classification	MAH timeline
SYN-PV-005 33 y; age recorded; sex unknown	Ciprofloxacin Tendon pain	Not reported / not documented	Not reported / not documented	Not reported / not documented	Structured seriousness field says 'Medically important'; clinical basis not described.	**SERIOUS***	15 calendar days
SYN-PV-008 36 y; male	Losartan Fatigue	Not reported / not documented	Not reported / not documented	Not reported / not documented	Source field says 'Not serious'; no medically important criterion described.	**NON-SERIOUS**	90 calendar days
SYN-PV-010 57 y; female	Atorvastatin Myalgia	Not reported / not documented	Structured seriousness field says 'Hospitalisation'; admission details absent.	Not reported / not documented	Hospitalisation is a qualifying criterion if inpatient admission/prolongation was due to the event.	**SERIOUS***	15 calendar days

Case / patient	Suspect product and reaction	Death / life-threatening	Hospitalisation	Disability	Other medically important event	Final classification	MAH timeline
SYN-PV-013 75 y; female	Ciprofloxacin Tendon pain	Not reported / not documented	Not reported / not documented	Not reported / not documented	Structured seriousness field says 'Medically important'; clinical basis not described.	**SERIOUS***	15 calendar days
SYN-PV-024 79 y; age recorded; sex unknown	Losartan Fatigue	Not reported / not documented	Structured seriousness field says 'Hospitalisation'; admission details absent.	Not reported / not documented	Hospitalisation is a qualifying criterion if inpatient admission/prolongation was due to the event.	**SERIOUS***	15 calendar days

* Serious classification is provisional pending source verification. The source dataset's structured seriousness field records 'Medically important' or 'Hospitalisation', but the narrative does not provide supporting clinical details. Do not silently convert missing details into confirmed findings.

4. Individual case assessments

SYN-PV-005 — Ciprofloxacin / tendon pain

A 33-year-old patient reported tendon pain 15 days after starting oral ciprofloxacin 500 mg twice daily. Treatment was stopped; recovery status was not achieved at follow-up. Dechallenge is unknown; rechallenge was not done; no concomitant medicines were reported.

Seriousness rationale: The structured seriousness field is 'Medically important'. This is an additional recognised seriousness category, separate from death, life-threatening status, hospitalisation and disability. However, the dataset does not explain whether the tendon event required intervention or otherwise jeopardised the patient. No death, immediate life threat, inpatient admission or persistent disability is documented.

Disposition: Provisional classification: SERIOUS (other medically important event), 15-day MAH reporting. Obtain clinical notes, functional impact, treatment/intervention, and admission status to substantiate the medically important designation.

SYN-PV-008 — Losartan / fatigue

A 36-year-old male reported fatigue 2 days after starting oral losartan 5 mg once daily. Treatment was stopped; the patient was recovering. Dechallenge was negative, rechallenge was not done, and paracetamol was listed as a concomitant medicine. The product reference marks fatigue as unexpected.

Seriousness rationale: The structured seriousness field is 'Not serious'. No death, immediate risk of death, inpatient hospitalisation, persistent/significant disability or medically important intervention is described. Unexpectedness does not itself make an event serious.

Disposition: Classification: NON-SERIOUS on available information. MAH reporting timeline: 90 calendar days. Continue follow-up for worsening, admission, significant incapacity or new information that could change seriousness.

SYN-PV-010 — Atorvastatin / myalgia

A 57-year-old female reported myalgia 21 days after starting oral atorvastatin 5 mg once daily. Treatment was stopped and the patient was recovered at follow-up. Dechallenge was unknown; rechallenge was negative; paracetamol was listed as a concomitant medicine.

Seriousness rationale: The structured seriousness field states 'Hospitalisation'. Inpatient hospitalisation or prolongation due to the adverse event is a seriousness criterion. However, the narrative does not state admission date, reason, duration, or whether the myalgia caused or prolonged the admission. These details must be verified.

Disposition: Provisional classification: SERIOUS based on the structured hospitalisation flag, 15-day MAH reporting. Validate actual inpatient admission and causal relationship to the event; an emergency visit alone is not equivalent to inpatient hospitalisation.

SYN-PV-013 — Ciprofloxacin / tendon pain

A 75-year-old female reported tendon pain 5 days after starting oral ciprofloxacin 500 mg twice daily. Treatment was stopped; the patient was not recovered at follow-up. Dechallenge was negative and rechallenge was positive; vitamin D was listed as a concomitant medicine.

Seriousness rationale: The structured seriousness field is 'Medically important'. A positive rechallenge is relevant to causality, not by itself to seriousness. The file does not document death, immediate life threat, inpatient hospitalisation or persistent/significant disability. The medically important basis and any intervention are not described.

Disposition: Provisional classification: SERIOUS (other medically important event), 15-day MAH reporting. Seek clinical documentation of severity, mobility/functional impact, intervention and rationale for medically important classification.

SYN-PV-024 — Losartan / fatigue

A 79-year-old patient (sex not specified) reported fatigue 3 days after starting oral losartan 500 mg twice daily. Treatment was stopped and the patient was recovering. Dechallenge and rechallenge were both positive; paracetamol was listed as a concomitant medicine.

Seriousness rationale: The structured seriousness field states 'Hospitalisation'. If inpatient admission or prolongation was caused by the event, the case meets a seriousness criterion. The narrative does not supply admission details. The reported dose is unusual for losartan and should be verified against the original prescription/medical record; do not assume it is correct.

Disposition: Provisional classification: SERIOUS based on the structured hospitalisation flag, 15-day MAH reporting. Urgently verify admission details and dose. Positive dechallenge/rechallenge may support causality but does not independently establish seriousness.

5. Criteria-level findings and data gaps

Criterion	Finding across selected cases	Assessment note
Death	No death is reported in any of the five selected records.	Not met on available information; absence of a report is not proof that it did not occur.
Life-threatening	No immediate risk of death is documented.	Not met on available information. Do not infer life-threatening status from symptom type or potential severity.
Inpatient hospitalisation / prolongation	Structured hospitalisation flag in SYN-PV-010 and SYN-PV-024.	Treat as serious provisionally; confirm inpatient admission/prolongation and event relationship. No details supplied.
Persistent/significant disability or incapacity	No persistent/significant disability is documented.	Not met on available information. Temporary fatigue or tendon pain alone does not establish disability.
Other medically important event	Structured 'Medically important' flag in SYN-PV-005 and SYN-PV-013.	Provisional serious classification; obtain medical rationale/intervention details. Positive rechallenge in -013 informs causality, not seriousness.

6. Recommended follow-up and quality checks

• Obtain source records for SYN-PV-010 and SYN-PV-024 to confirm whether true inpatient admission/prolongation occurred, dates, reason and relationship to the adverse event.

• Obtain the reporter's clinical rationale for 'Medically important' in SYN-PV-005 and SYN-PV-013, including treatment required, functional consequences and potential for serious harm.

• Verify the losartan dose in SYN-PV-024 (500 mg twice daily) with the original prescription and dispensing/administration record; document any correction without overwriting the source.

• Confirm patient identifiers, event onset/stop dates, outcome, relevant medical history, laboratory findings, treatment, and reporter contact details where available.

• Reassess seriousness whenever follow-up information is received. Upgrade promptly if a qualifying outcome is confirmed; downgrade only when supported by documented evidence and applicable procedures.

• Assess causality and expectedness separately from seriousness. An unexpected event may be non-serious; a known/expected event may still be serious.

7. Overall conclusion

Of the five selected cases, four are provisionally classified as Serious (SYN-PV-005, -010, -013 and -024) and one as Non-Serious (SYN-PV-008). The two hospitalisation cases and two medically important cases require source-data validation because the structured seriousness field is not supported by sufficient narrative detail. No death, life-threatening outcome or persistent/significant disability is documented in the selected records. Apply 15 calendar days for serious and 90 calendar days for non-serious MAH reporting under the cited Indian CDSCO guidance, calculated from receipt of information.

8. References

1. Central Drugs Standard Control Organisation (CDSCO). Guidance Document on Post-Marketing Surveillance / Periodic Safety Update Reports. 2024. Section 2.7: reporting timelines (serious AEs within 15 calendar days; non-serious AEs within 90 calendar days). Available at: https://cdsco.mohfw.gov.in/opencms/resources/UploadCDSCOWeb/2024/PSURFiles/Guidance_Documentpsur2.pdf

2. Indian Pharmacopoeia Commission. Pharmacovigilance Programme of India (PvPI): Frequently Asked Questions. Definition of serious ADR/SAE and encouragement to report suspected ADRs. https://ipc.gov.in/PvPI/faq.html

3. US Food and Drug Administration. Expedited Safety Reporting Requirements for Human Drug and Biological Products. Seriousness criteria and medically important events (used as supporting general regulatory definition, not as the Indian reporting timeline). https://www.fda.gov/science-research/clinical-trials-and-human-subject-protection/expedited-safety-reporting-requirements-human-drug-and-biological-products

Training-use disclaimer: The source pack itself states that all cases are fictional and intended for educational analysis only, not clinical care or regulatory submission. This report is a training assessment, not a validated ICSR submission or legal regulatory advice. Final regulatory decisions must be made by the responsible qualified pharmacovigilance professional using current applicable product-specific and jurisdictional requirements.